NCCP 00322 · VERSION 4

Everolimus + exemestane

Both tablets are taken every day continuously. There is no planned week off. A four-week blood-test review is a monitoring interval, not a stop-and-restart treatment cycle.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Mouth ulcers/stomatitis

☐ Non-infectious pneumonitis

☐ Reduced blood counts

☐ Renal impairment/proteinuria

☐ Joint and muscle pain

☐ Vaginal dryness/sexual symptoms

☐ Fatigue

☐ Infections, including opportunistic infection

☐ High glucose and abnormal lipids

☐ Rash, fatigue and diarrhoea

☐ Impaired wound healing and radiation-related toxicity

☐ Hot flushes and sleep disturbance

☐ Bone thinning and fractures

If everolimus is interrupted or reduced for toxicity, NCCP says exemestane may continue; the treating team must confirm the plan for each medicine. Do not stop both automatically or restart everolimus yourself. Everolimus can increase toxicity around radiotherapy and impair wound healing: agree a specific plan before radiotherapy or surgery. This NCCP indication is postmenopausal. If pregnancy remains possible or menopause is uncertain, ask the team to confirm endocrine status and suitable contraception. Everolimus product information advises highly effective contraception during treatment and for eight weeks after it ends; pregnancy and breastfeeding are NCCP exclusions.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: FBC, renal and liver profile, urea/BUN, urinary protein, blood glucose, cholesterol and triglycerides. Confirm the endocrine indication, review lung symptoms and infections, and check all medicines. Assess bone/fracture risk for the exemestane component. FBC, renal/liver profile, cholesterol and triglycerides every four weeks or as clinically indicated. Check fasting glucose as clinically indicated, with closer monitoring if diabetes or another medicine increases risk. Ask about mouth soreness, infection, cough/breathlessness and daily function at reviews.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.